Your Special Needs Co-Parenting Report

Prepared for A. Sample Parent · 10-principle framework

Overall rating: Building Progress

You have real strengths to build on, alongside a handful of principles that would benefit from focused attention. This report highlights the highest-leverage next steps for supporting your child across two homes.

Your strengths

- Understand & Share Your Child's Diagnosis and Needs

Build shared knowledge of the diagnosis and its daily impact.

- Practice Compassion and Patience

Approach disagreements with empathy; keep the child at the center.

- Prioritize Consistent Routines & Predictability

Keep core routines the same in both homes to reduce anxiety.

Priority focus areas

- Maintain Open, Child-Focused Communication (Needs Improvement)

Why it matters: Conversations escalate before important information is shared, which can put continuity of care at risk.

First step: Move all non-emergency communication to a written, timestamped channel.

- Build an Individualized Parenting Plan (Fair)

Why it matters: The plan feels like a generic template in places - it doesn't fully reflect your child's regulation patterns.

First step: Rewrite one section to name the specific supports your child needs (sensory, transition, medication).

- Align on Core Expectations (Fair)

Why it matters: Medication schedules or behavioral expectations differ enough to confuse your child.

First step: Ask the prescribing provider to give both parents the same written protocol.

- Manage Transitions Smoothly (Fair)

Why it matters: Your child shows anxiety around exchanges more often than you'd like.

First step: Move exchanges to a neutral, low-stimulation location.

Communication toolkit

Because Principle 3 (Maintain Open, Child-Focused Communication) is currently Fair or Needs Improvement, these tools are highlighted for you.

- Use BIFF messages

Brief, Informative, Friendly, Firm. Keep each message to 4 sentences or fewer.

- Move to a written channel

Use a shared parenting app or dedicated email thread so therapy, medication, and school updates are timestamped.

- Weekly child-focused check-in

10 minutes, same day/time, fixed agenda: therapy progress, medication, school, one 'what's working'.

- Delay before responding

Wait 4 hours before replying to a charged message. Draft, save, revise, then send.

- Facts, not feelings, in writing

Move emotional processing to a therapist or trusted friend. Keep co-parent communication practical.

Principle-by-principle detail

Principle 1: Build an Individualized Parenting Plan

Rating: Fair

Tailor the schedule to your child's needs rather than a generic template.

Findings

- The plan feels like a generic template in places - it doesn't fully reflect your child's regulation patterns.

Recommendations

- Rewrite one section to name the specific supports your child needs (sensory, transition, medication).
- Ask your child's therapist to help translate clinical needs into schedule language.

Principle 2: Prioritize Consistent Routines & Predictability

Rating: Good

Keep core routines the same in both homes to reduce anxiety.

Findings

- Most core routines in your home are consistent.

Recommendations

- Pick one routine (bedtime, mealtime) to align with the other home.

Principle 3: Maintain Open, Child-Focused Communication

Rating: Needs Improvement

Schedule dedicated, child-focused communication; log details in shared tools.

Findings

- Conversations escalate before important information is shared, which can put continuity of care at risk.

Recommendations

- Move all non-emergency communication to a written, timestamped channel.
- Consider a Parenting Facilitator experienced with special-needs coordination.

Principle 4: Align on Core Expectations

Rating: Fair

Agree on core rules even when parenting styles differ.

Findings

- Medication schedules or behavioral expectations differ enough to confuse your child.

Recommendations

- Ask the prescribing provider to give both parents the same written protocol.
- Agree on a shared discipline approach for meltdowns - even a one-page document helps.

Principle 5: Understand & Share Your Child's Diagnosis and Needs

Rating: Excellent

Build shared knowledge of the diagnosis and its daily impact.

Findings

- You have a strong working knowledge of your child's diagnosis and support needs.

Recommendations

- Share updated diagnostic reports with all providers and the other parent.

Principle 6: Coordinate Education & Therapy

Rating: Good

Work together on IEP/504 plans, school placement, and therapy carryover.

Findings

- Coordination on education and therapy usually works.

Recommendations

- Ask the school to add both parents to all official communications.

Principle 7: Manage Transitions Smoothly

Rating: Fair

Plan exchanges to minimize sensory and emotional stress.

Findings

- Your child shows anxiety around exchanges more often than you'd like.

Recommendations

- Move exchanges to a neutral, low-stimulation location.
- Use transition warnings and a visual schedule 24 hours before each exchange.

Principle 8: Be Flexible and Adaptable

Rating: Good

Adjust the plan as the child's needs, abilities, and preferences evolve.

Findings

- You're generally open to adjusting the plan when things change.

Recommendations

- Set a formal 6-month plan review to catch drift proactively.

Principle 9: Build a Support Network

Rating: Good

Engage therapists, educators, and other professionals as partners.

Findings

- You have professionals to consult when you're stuck.

Recommendations

- Consider joining one parent-support group for continuity between visits.

Principle 10: Practice Compassion and Patience

Rating: Excellent

Approach disagreements with empathy; keep the child at the center.

Findings

- You approach disagreements with empathy and keep your child at the center.

Recommendations

- Continue modeling self-regulation for your child.

Recommended next steps

- Move all non-emergency communication to a written, timestamped channel.
- Rewrite one section to name the specific supports your child needs (sensory, transition, medication).
- Ask the prescribing provider to give both parents the same written protocol.
- Move exchanges to a neutral, low-stimulation location.

This report is an educational reflection tool from SA CoParents. It is not a diagnostic evaluation, custody evaluation, or legal advice. Findings are based on your own self-assessment answers on a 5-point scale across the 10-principle special-needs co-parenting framework.